

إسم المريض ربيعاً : هيام جابر ابراهيم محمد
المحافظة : القاهرة الحي / المدينة : البساتين
رقم البطاقة : 27504210103546 رقم التليفون : 01011831186
العمر(تاريخ الميلاد) : 21-04-1975 المهنة : ربه منزل
العنوان : البساتين
تاريخ تسجيل الإستمارة : 06-10-2026

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] **قسم العلاج :** الاورام

التعليق على التشخيص : -----

GR1-E

جوسٹا فیکٹری

روستائی پلاننگ

لڈی شاپ

جوسٹا فیکٹری

صحة الشباب والمرقة

اللجنة و مدير

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس

وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- / 2 ----- / 2 ----- / 2.1 ----- / 2.1

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج : --

ملاحظات : ---

قسم :

نَفَقَات :

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: هيام جابر ابراهيم محمد National ID: 27504210103546	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Bilateral mammogram and breast ultrasound
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



Name: هيام جابر ابراهيم محمد			
MR. : 3161			
Detailed Medical Report			
		السيد	
تحتية طبية وبعد,,			
		إيماء للطلب المقدم من	
بشأن تقرير طبي للمريض			
رقم طبي:		بقسم:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الخروج :		تاريخ وساعة الدخول :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient has premenopausal right-sided breast cancer, clinical stage T4B N1, with pathology showing estrogen receptor negative (ER-VE), progesterone receptor negative (PR-VE), and HER2 3+ positivity. The disease was diagnosed on 10 August 2024. Surgical management included right breast skin-sparing mastectomy (RT SSM) with sentinel lymph node biopsy (SLN), with pathology revealing no residual tumor in the breast (YPT0) and no nodal involvement (YPNO 0/7). The patient was planned for HERCEPTIN-based therapy. At follow-up, the patient remains on HERCEPTIN therapy, having completed the most recent cycle and scheduled for follow-up visits every three months.			
تحريراً في ,			
التاريخ :		19-09-2026	
الوقت :		dina makram	
		الطبيب المعالج :	
يعتمد ,			



A-Patient ID:

1- Name:

النام جابر إبراهيم محمد

Sex:

MDT form

3- Medical number:

Female

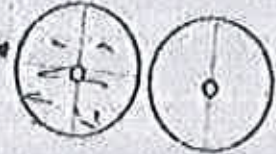
83177945

Old Med:

Old Med:

4731/2023

Patient Age: 49 Y



Family History

Family History
-VE FH

Phone

Phone

01029795843

BNI

BNI

marital status

Marital Status
WIDOW

NRS Score

NRS Score

GROUP

GROUP

Co morbidity

Co Morbidity
NADMenopausal
statusMenopausal Status
POSTMENOPAUSAL

Initial STAGE

Initial STAGE
ADVANCED

REASON OF MDI

REASON OF MDI
NEW

Date of MDI

Date of MDI

08/01/2024

Breast Imaging:

RT LQ 5 o'clock solid mass 5.5*2.6 cm w nipple retraction
skin dimpling 2 pathological LNS

Mets Workup:

pelvic - abdomen U/S: free
CT chest: free

BX

IDC

ER

0/8

PR

2/8

Her 2 Neu

3

SISH

KI67
50%

EX COMMENT:

GIII w focal DCIS

EX COMMENT:

Her 2 Neu COMMENT:

SISH COMMENT:

T

CT4B

N

1

M

0

Sonomammography

PATIENT HISTORY
FOR ECHO, LABS
BONE SCAN
CT ABD AND PELVIS

NOT OUTSIDE & PLAN

NACT ACX4 TAXOTERE HERCEPTIN PERJETA X 4 THEN RT MASTECTOMY

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

Date: 17/1

SURGERY

SURGERY

Date: 17/1

Psychiatric

Psychiatric

RADIOLOGY

PATHOLOGY

Nutrition

RADIOLOGY

RADIOLOGY

PATHOLOGY

PATHOLOGY

Nutrition

Nutrition



Ministry of Health and Population
Ministry of Health and Population

الوزارة العامة للصحة والسكان
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : هادي جابر بن اكرم محمد

Sex:

3- Medical number:

Female

83177945

Old Med:

Date:

4/31/2023

2-ID number:

Patient Age: 49 Y

Family history:

Family history:

-ve

Phone:

Phone:

01079705843

BMI:

BMI:

REASON OF MDT:

REASON OF MDT:

Marital status:

Marital status:

-ve

HRS Score:

HRS Score:

-ve

GROUP:

GROUP:

Date of MDT:

Date of MDT:

29/09/2024

Comorbidity:

Comorbidity:

-ve

Menopausal status:

Menopausal status:

-ve

Initial STAGE:

Initial STAGE:

-ve

Clinical data

Breast Imaging:

RT LIQ 5 o'clock solid mass 5.5*2.6 cm c/ nipple retraction
skin dimpling 2 pathological LNS

Mets Workup:

CTCAP FREE
BONESCAN FREE

BX:

ER:

PR:

Her 2 Neu:

SISH:

Ki67:

IDC:

0/8

2/8

3

GI11 & total DCIS

11 (LUNGS)

11 (LUNGS)

11 (LUNGS)

11 (LUNGS)

50

T:

N:

M:

4b

1

0

Sonomammography

PATIENT HISTORY

RECEIVED NACT ACX4 TAXOTEREX4 , NO ANTI HER2 DUE TO REDUCTION OF
EF FROM 35% to 65% with treatment RT SM+SLNB: free margins
LN 0/7

MDT OUTCOME & PLAN

MDT Outcome & Plan:

For Herceptin with very close follow up of EF.
PORT

SIGNATURE

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

RADIOLOGY

RADIOLOGY

SURGERY

SURGERY

PATHOLOGY

PATHOLOGY

Psychiatric

Psychiatric

Nutrition

Nutrition



Patient Name : الأستاذ / هيام جابر إبراهيم محمد الجابري
Patient No. : 27504210103546
Path No. : 4220/23
Sex : Female
Age : 48 Y
Referring Doc. :

Request No. : 13006972
Request Date : 28/11/2023
Verification Date : 16/12/2023
Financial Category : أورام دار السلام - نفقة الدولة
Department : Out Patient
IHC No. :
Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 27/11/2023, 12:40 pm.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- IPT (Positive control showed appropriate reactivity):
ER: No nuclear reaction in the tumor cells (Allred score 0/8).
PR: Weak nuclear reaction in 1% of tumor cells (Allred score 2/8).
Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
Ki67: 50% nuclear reaction of tumor cells.

Conclusion:

Right breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade III with focal DCIS and marked lymphocytic infiltration.
ER: Negative (score 0/8).
PR: Negative (score 2/8).
Her2: Positive (score 3+).
Ki67: Positive in 50% of tumor cells.

Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, >66%).

Verified by :

Rana fathy

Printing Date : 18/12/2023 09:20:22

Page 1 of 2

Printed By : marwa mohamed yousse



Patient Name: الأستاذة / هيام جابر إبراهيم محمد الجابري
Patient No.: 27504210103546
Path No.: 4220/23
Sex: Female
Age: 48 Y
Referring Doc.:

Request No.: 13006972
Request Date: 28/11/2023
Verification Date: 16/12/2023
Financial Category: أورام دار السلام - نفقة الدولة
Department: Out Patient
Registered by: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature:
Dr. Rana Fathy
Dr. Menan Hossem

Verified by:
Rana fathy

Printing Date : 18/12/2023 09:20:22

Page 2 of 2

Printed By : marwa mohamed yousse

Room No. (4) - First Floor Building No. 987 - Nile Corniche - Misr El Qadima - Cairo, Egypt
غرفة رقم (4) - الدور الأول - مبنى رقم (987) - قورنيش النيل - مصر القديمة - القاهرة - مصر

Patient Name : الأستاذة / هيام جابر إبراهيم محمد الجابري
Patient No. : 27504210103546
Path No. : 4220/23
Sex : Female
Age : 48 Y
Referring Doc :
Request No. : 13006972
Request Date : 28/11/2023
Verification Date : 16/12/2023
Financial Category : نفقة الدولة - اورام دار السلام
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Right breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 27/11/2023, 12:40 pm.

Gross Examination:

• Three grayish white tissue cores up to 1 cm, totally submitted.

Microscopic Examination:

- Histological type: Invasive duct carcinoma .
- Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+3+2) Grade III .
- Ductal carcinoma in situ: Focal .
- Perineural invasion: No.
- Marked lymphocytic infiltration .

Diagnosis:

Right breast mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade III with focal DCIS and marked lymphocytic infiltration .

For further IHC (ER, PR, Her2, Ki67).

Signature

Dr. Rana Fathy

Dr. Menan Hossam

Verified by :

Rana fathy

Printing Date : 18/12/2023 09:20:36

Page 1 of 1

Printing By : marwa mohamed youss

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Misr El Qadema - Cairo, Egypt



Name: haiaam gaber ibrahim Mohamed Date: 27/11/2023

Bilateral mammogram and US of both Breasts

Clinical summary: right breast palpable lump

FINDINGS :

LEFT BREAST:

- Scattered fibroglandular mammary pattern (ACR B)
- No speculated masses or pathological microcalcifications .
- Complementary US :No solid or cystic lesions
- Normal skin / nipple / areolar complex.
- Nonspecific preserved axillary nodes.

RIGHT BREAST:

- Scattered fibroglandular mammary pattern (ACR B)
- LIO asymmetry ,ill defined dense lesion ,distortion and microcalcifications
- U/S shows irregular hypoechoic solid lesion seen at 5 o'clock measuring 55x26 mm with penetrating vascularity detected upon Color Doppler application /it is reaching to skin causing dimpling and reaching to nipple causing retraction
- Enlarged two pathological lymph node .they measures 17x8mm cortical thickening 4.5mm and 18x9mm with cortical thickening 5mm

IMPRESSION :

Right BREAST: (BIRADS 5) malignant looking solid lesion .for biopsy

Left BREAST: (BIRADS 1= Normal)

(biopsy was taken in same sitting)

Best regards,

Dr. Marwa M.Hassan



Patient Name: Hayam Gaber Ibrahim Mohamed	Date : 08/01/2024
Exam : C.T ABDOMEN & PELVIS	Age : 48 Year(s)
Medical ID: 4731/2023	Pat. ID: 45173

**NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED; -**

1. Chest:

Correlation with previous study dated 18th of Dec, 2023

- Still seen right breast lower inner quadrant speculated mass with extension into retroareolar region, it measures about 3.8x2.3 cm.
- Still seen multiple right axillary level I enlarged lymph nodes, the largest measures about 2x1.3 cm.
- Stable right lower lung lobe nodule at posterior segment measuring 4 mm.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- No gross cardiac abnormality. No pericardial effusion detected.
- Normal non-enhanced appearance of the thoracic aorta and the great vessels.
- No worrisome lytic or sclerotic bony lesions.
- Upper abdominal cuts are unremarkable.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.



Patient Name: Hayam Gaber Ibrahim Mohamed	Date : 08/01/2024
Exam : C.T ABDOMEN & PELVIS	Age : 48 Year(s)
Medical ID: 4731/2023	Pat. ID: 45173

- No abdominal or pelvic masses or collections.

3. The scanned skeleton:

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- *Right breast malignant looking mass lesion with pathological ipsilateral axillary lymph nodes as described.*
- *Stable tiny right pulmonary nodule likely benign in nature for follow up.*
- *No definite hepatic metastatic deposits.*

Dr. Mohammed Mohsen, MSc.

Signature Prof. Mohamed Gomaa
09/01/2024 08:41 م

Dr. M. Gomaa, M.D.

05 Aug 2025

صالح جابر

ECHO REPORT

Name:

Date: ٢٦ / 9 / 2025

EF	55%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.9	3.5-5.8cm	AO.R	2.5	2-3.7cm
LVESD		2-4cm	LA	3.5	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

- LV dimensions ✓ *normal*
- LV wall thickness ✓ *normal*
- LV diastolic function *DD grade I*
- SWMA ✓ *normal*
- LA and Ao root dimensions ✓ *normal*
- Mitral Valve ✓ *normal*
- Aortic valve ✓ *normal*
- Tricuspid valve *TURB*
- Pulmonary valve ✓ *normal*
- Right side and systolic function ✓ *normal*
- Intracardiac masses and effusion ✓ *normal*

Dr /
[Signature]